



Acute Asthma Exacerbation

- Check → BP, HR, RR, O₂, PEFR.
- Assess → Severity (Moderate, Severe, Life threatening, Near Fatal).

ASTHMA CLINICAL SCORE (PRAM)				
Signs	0	1	2	3
Suprasternal Indrawing (tracheal tug)	Absent		Present	
Scalene retractions (Use of accessory Muscles)	Absent		Present	
Wheezing	Absent	Expiratory only	Inspiratory and expiratory	Audible without stethoscope/silent chest with minimal air entry
Air entry	Normal	Decreased at bases	Widespread decrease	Absent/minimal
SpO ₂ in room air	≥94%	90 – 93%	≤89%	
Severity		Asthma Clinical Score		
Mild		0 - 4		
Moderate		5 - 8		
Severe		9 - 12		
Impending respiratory failure		12 + presence of lethargy, cyanosis, decreasing respiratory effort, and/or rising pCO ₂		

Give:

- **O₂ driven Nebulization if hypoxemia** (Target SpO₂ 94-98%)
- **Steroid** in adequate doses to all patients with an acute asthma attack (Oral preferred to IV/IM)
- Continue Prednisolone **Adult: 40-60mg daily** or **Pediatrics: 1 mg/kg/day** until recovery (5-7 days)
- Assess response → **Repeat Ventolin every 20 min for 1 hour if indicated**
- If poor response → **Refer to medical casualty by ambulance**

Medication	Adult dose	Pediatric dose	Age
Salbutamol (Ventolin Inhaler with spacer OR Nebulizer if hypoxemia)	Inhaler (MDI + spacer): 4-10 puffs	Inhaler (MDI + spacer): 2-6 puffs 2-10 puffs	Infants and children < 5 y Children > 6 y
	Nebulizer: 5mg = 1 ml	Nebulizer: 2.5mg = 0.5ml	
Ipratropium bromide (Atrovent Nebulizer) → if severe or no response	0.5mg = 2 ampules	0.25mg = 1 ampule	

Medication	Adult dose	Pediatric dose	Age
Oral prednisolone	40-60 mg	(1 mg/kg/day) 20 mg/d 30-40 mg/d	2-5 years > 5 years
Or IV/IM Hydrocortisone (off-label dose)	100 mg	50mg 100 mg	<2 years 2-5 years